

University Cancer Institute - Hematology Clinic

PATIENT: ADAMS, Sandra
MRN: AML032
DOB: 28-Feb-1968
DATE OF VISIT: 06-Mar-2024

SUBJECTIVE:

Ms. Adams is a 56-year-old female with IDH2-mutated AML, here for a routine follow-up visit. She reports feeling very well. Her energy levels are "back to about 80% of my normal," and she has been able to return to part-time work as a librarian. She has no complaints of fever, chills, unusual bleeding, bruising, or bone pain. She is tolerating her oral maintenance therapy without significant side effects, noting only occasional mild nausea that is well-controlled with prochlorperazine.

PAST MEDICAL HISTORY:

- *Acute Myeloid Leukemia with IDH2 R140Q mutation (diagnosed 25-Jan-2023).*
- *Deep Vein Thrombosis (DVT) of the left lower extremity (Feb 2023), currently on Apixaban.*
- *Hypothyroidism, well-controlled on levothyroxine.*

TREATMENT HISTORY:

- **Induction:** *Standard "7+3" (Daunorubicin/Cytarabine), started 01-Feb-2023. Complicated by neutropenic fever and DVT. Achieved complete remission (CR).*
- **Consolidation:** *Completed 2 cycles of high-dose cytarabine (HiDAC), finished in June 2023.*
- **Maintenance:** *Started on Onureg in July 2023. Currently in Cycle 8 of maintenance.*

OBJECTIVE:

Vitals: *T 37.0°C, HR 66, BP 118/72 mmHg, SpO2 99% RA.*

General: *Pleasant, well-appearing female in no distress.*

Labs (06-Mar-2024):

- **CBC:** *WBC 5.1 K/uL, ANC 3.2 K/uL, Hgb 12.9 g/dL, Plt 201 K/uL.*
- **CMP:** *All WNL. Bilirubin, total 1.3 mg/dL (stable, known side effect of Onureg), Direct Bili 0.2. LFTs are normal.*
- **IDH2 Mutant Allele Frequency (by ddPCR on peripheral blood):** *Not Detected.*

ASSESSMENT & PLAN:

Ms. Adams is a 56-year-old female with ELN 2022 intermediate-risk AML who remains in a deep, molecular complete remission.

1. **AML Status:** *Excellent. She continues to have a superb response to therapy. The absence of a detectable IDH2 mutation in her blood is a very strong positive prognostic indicator. We will continue with the current plan.*
2. **Maintenance Therapy:** *She is tolerating Onureg well. The asymptomatic, indirect hyperbilirubinemia is an expected and benign side effect.*
 - **Plan:** *Dispensed prescription for next 3 months.*

3. **Anticoagulation:** *Her DVT was clearly provoked by her central line and chemotherapy. Per hematology guidelines, she has completed an adequate duration of anticoagulation. After discussion of risks/benefits of continuation, we will plan to stop.*
 - **Plan:** *Discontinue Apixaban. Counseled patient on signs/symptoms of recurrent VTE.*
4. **Health Maintenance:** *Reviewed age-appropriate cancer screenings. She is due for a mammogram.*
5. **Follow-up:** *RTC in 3 months for clinical assessment and labs. Will continue to monitor MRD status via peripheral blood every 6 months.*

Overall, her prognosis is very good. She is a model patient for the success of targeted maintenance therapy post-chemotherapy.

Dr. Eleanor Vance, MD
Hematology Service